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Pilot Evaluation of the Families Make the Difference Parenting Program With Arab Families Residing in Berlin, Germany

Sascha Hein* 

Freie Universität Berlin,
 GERMANY / Yale Child Study
 Center, USA

Isabell Schuster 

Freie Universität Berlin,
 GERMANY

Israa Alassi 

International Rescue
 Committee, GERMANY

Aumnia Mohamad 

International Rescue
 Committee, GERMANY

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Abstract: We examined the impact of the program “Families Make the Difference” (FMD) on well-being, feelings of distress, parental stress, parenting, co-parenting, paternal engagement, and several exploratory outcomes of Arab families in Berlin, Germany. FMD is a parent support program designed to assist families with refugee and migration backgrounds as they navigate life in Germany, where they often face challenges such as uncertain residency status, economic hardship, and the loss of their social and familial networks. In addition to the main effects on key outcomes, we also hypothesized that the changes in social support relate to the changes in parent characteristics (e.g., parental stress) and parenting behaviors. One hundred thirty-five caregivers participated in FMD and completed questionnaires before and after the program. We found small but statistically non-significant changes in most outcome measures over time. The only statistically significant finding indicated that female participants reported a decrease in parental stress over time, whereas male participants, on average, reported an increase. The results also suggest that increases in the perceptions of social support over time were associated with reductions in parental stress and feelings of distress and with increases in sensitive parenting, a composite measure of responsive caregiving, paternal engagement with the child and his spouse, and co-parenting. The findings imply that parenting programs with Arab families should be tailored to the needs of male and female caregivers. Future studies should examine the role of social support as a contributing factor to the benefits of parenting programs.

Keywords: Arab families, fathers, parental stress, parenting program, social support.

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Introduction

Group-based parenting programs can potentially improve short-term parental psychological well-being (e.g., reductions in perceptions of stress, and improvements in confidence; Bennett et al., 2013; Lakkis et al., 2020; Ponguta et al., 2020), parenting knowledge and practices (Jeong et al., 2021; Ponguta et al., 2020), and child cognitive, language, and social-emotional outcomes (Jeong et al., 2021; Lakkis et al., 2020). Such programs are offered to groups of parents, use a standardized or manualized curriculum that combines basic behavioral strategies with cognitive strategies, and are based on a theory of change to address parental functioning and behaviors to manage children's behaviors or to improve overall family functioning and responsive parent-child relationships (Bennett et al., 2013), particularly in the first three years of life and in low- and middle-income countries (Jeong et al., 2021). Historically, mothers were the primary beneficiaries of such programs. However, a growing corpus of evidence suggests that including fathers in parenting interventions is an added value for promoting positive parenting, co-parenting, and father-child relationships (Hein et al., 2020; Henry et al., 2020; Holmes et al., 2020; Panter-Brick et al., 2014). Nevertheless, several aspects must be addressed to keep fathers involved and engaged (e.g., offering father-only groups, adapting programs to incorporate

* Corresponding author:

Sascha Hein, Freie Universität Berlin, Berlin, Germany, and Yale Child Study Center, New Haven, CT, USA. ✉ sascha.hein@fu-berlin.de



father preferences) for a program to yield the desired effects (de Santis et al., 2020; Gonzalez et al., 2023; Lundahl et al., 2008).

Effects on Parent-Child Outcomes

Parenting programs can effectively reduce parental stress (Cooley et al., 2014) because of their capacity to improve parent-child relationships (Li et al., 2021). In a recent component network meta-analysis, Tehrani et al. (2024) found that combining behavior management strategies (e.g., positive reinforcement techniques such as praise and rewards and non-violent disciplining techniques such as ignoring and time-outs) with approaches to enhance parent-child interaction was the most effective approach to reducing parental stress for parents of children with disruptive behaviors. Among mothers and fathers with children aged 0-12 years, parenting stress is associated with several modifiable factors, such as social support and maternal depression (Fang et al., 2024). Thus, promoting social support through participation in parenting programs has the potential to act as a protective factor in reducing parenting stress.

Social Support

Álvarez et al. (2021) found support for the role of social support in parenting programs. The authors examined the associations between different dimensions of social support and changes in parents with 0-5-year-old children who participated in a group-based parenting program. They differentiated formal sources of support (e.g., professional-to-client exchanges that involve institutions and professionals such as social workers and teachers) and informal support (e.g., provided by the parents' personal networks, including family members, friends, and neighbors), as well as different types of support including emotional and tangible support. The authors found that formal support was associated with reductions in parenting stress. In contrast, informal support was associated with reductions in the beliefs in corporal punishment but not with reductions in parenting stress. Other studies have found that more emotional support (the number of people in one's social network and the satisfaction with receiving emotional support) was associated with less parenting stress and more playful mother-child interactions (Ayala-Nunes et al., 2016; Berry & Jones, 1995; Zhang et al., 2016). In addition to bivariate associations, other studies have found support for the mediating role of social support. For instance, Liu et al. (2020) surveyed Chinese immigrant parents in the United States. They found that lower levels of perceived parental stress were associated with higher levels of social support, which was, in turn, related to higher levels of positive parenting (i.e., parents' self-ratings on positive child-rearing practices, attitudes, and beliefs; Liu et al., 2020). Thus, it is plausible that social support, parental stress, and parenting behavior are associated, particularly among families participating in a structured parenting program. These associations may be particularly robust for parenting programs that aim to match the support that the program activities can provide, including interactions with socially or experientially similar others, with the socioemotional needs (e.g., need for advice on child-rearing, formal support in navigating the child-care system) of parents (Thoits, 1995).

Parenting Programs for Migrant Families

Previous research has documented the effects of family interventions and parent support programs for refugee, forcible displaced, and migrant families that are comparable to those for the general population, particularly concerning positive parenting behaviors and attitudes, parent mental health, and family well-being, improved parent-child relationships, reduced parenting stress, and child psychosocial and developmental outcomes (Gillespie et al., 2022; Hamari et al., 2022; Lee & Kim, 2022; Lim et al., 2023). These programs promote engagement and retention of parents in the program activities and desired effects when they are culturally tailored to the needs and experiences of migrant parents (Hamari et al., 2022; Rao et al., 2023).

Masarik et al. (2022) expanded the family stress model (Masarik & Conger, 2017) to hypothesize that specific sources of resettlement stress may negatively impact parent and youth psychological distress (e.g., depression, anxiety, and hostility), which may, in turn, increase family distress (e.g., family conflict, harsh parenting) and, ultimately, impede child and youth cognitive and psychosocial adjustment. Children and families who were displaced and resettled into new communities may experience unique environmental, child-specific, and parent-specific challenges before and during the resettlement process, including unaddressed basic needs, exposure to conflict, acculturation differences, and discrimination (El-Khani et al., 2016; Rao et al., 2023). Parents' extended family social support networks, neighborhood cohesion, and connection with other families who resettled to a new community may reduce resettlement stress and disrupt its negative consequences (Masarik et al., 2022). Resettlement and (forced) displacement are major life events for a family. According to the social support deterioration deterrence model (Norris & Kaniasty, 1996), the experience of a major life event (e.g., a natural disaster) may lead to support mobilization and an increase in perceived social support, which reduces long-term distress. It is plausible to assume that families resettling to a new community actively seek to participate in a parenting program to mobilize formal and informal social support in addressing basic (e.g., seeking advice on child care and support in balancing multiple responsibilities) and socioemotional needs (e.g., understanding and responding to their child's behaviors and emotional needs). Reduced access to family support, adjustments in parenting roles due to resettlement, learning a new language, and securing employment are salient

challenges for refugee fathers (Bulford et al., 2022). When seeking support to manage these challenges, refugee parents, while separated from their traditional family and cultural support, prefer emotional and informational support from peers in their cultural community and culturally sensitive service providers (Stewart et al., 2017). This finding aligns with the assumption that families seek (parenting) support from experientially similar groups in which they feel socially accepted (Stewart et al., 2018; Thoits, 1995).

The Present Study

Much research has shown that group-based parenting programs can improve parental and child outcomes (e.g., Bennett et al., 2013; Jeong et al., 2021; Lakkis et al., 2020; Ponguta et al., 2020). Studies have also documented the importance of social support as a malleable factor that impacts program outcomes. However, research has focused mainly on the general population, often overlooking the unique needs of diverse groups, such as migrant and forcibly displaced families. Therefore, the International Rescue Committee (IRC) Germany has developed and implemented the Families Make the Difference (FMD) project, which is designed to assist families with refugee and migration backgrounds as they navigate life in Germany, where they often face challenges such as uncertain residency status, economic hardship, and the loss of their social and familial networks. These stressors can overwhelm parents, affecting their ability to care for their children effectively. Therefore, the FMD program aims to address these challenges by empowering parents and caregivers through native language meetings, providing a supportive environment to discuss their struggles, share experiences, and learn valuable coping strategies.

FMD offers meetings in different languages, ensuring accessibility for various refugee communities. The meetings are free of charge, and childcare is provided when necessary, allowing parents to engage fully. Each group meets eight to ten times, focusing on strengthening families by equipping parents and caregivers with knowledge on various topics such as stress management, early childhood development, and their children's physical and cognitive growth. The meetings also include activities that promote parent-child interaction. Sessions are held both online and in person. These meetings are held by IRC-trained facilitators who seek to create a safe and respectful space for open dialogue. The facilitators are individuals who speak one or more of the native languages of the participating caregivers as their native language and who have an interest in empowering families. The facilitators are typically well-connected to different communities in their area of residence and self-organize their involvement in local organizations and centers that offer advisory and counseling services. Not all facilitators have a formal post-secondary education in education sciences, migration studies, or psychology. However, they are experts in the lived experiences and realities faced by individuals with experiences of (forced) displacement or migration. The IRC offers workshops for all first-time group facilitators, during which the permanent IRC project staff trains the facilitators on the content and modules of the handbook. The facilitators familiarize themselves with the modalities and activities of the program, and administrative and logistic questions are discussed. IRC also offers regular meetings for facilitators to provide feedback and engage in discussions with other facilitators and the IRC staff. Group facilitators also have the option to invite experts as co-facilitators for specific topics and sessions.

During the sessions, facilitators work with parents to develop strategies for managing stress and addressing changes in family structures and roles that often arise after fleeing their home country. Community discussions and shared experiences also provide access to valuable information about available resources and additional support services for families. Two handbooks were developed in 2019 and 2020: one for parents of young children (0-11 years) and the other for parents of teenagers (12-18 years). Each handbook featured eight units on topics such as caring parenting, stress management, understanding children's needs, and empathetic communication. These handbooks served as essential resources, equipping facilitators to conduct meetings with parents and caregivers and equipping them with practical tools for daily life. FMD's content and delivery align with several critical facilitating factors to engagement in parenting programs, including delivery by trusted people in a convenient setting (Mytton et al., 2014). It is, in essence, an innovative social care service that builds relationship-level protective factors, including parenting approaches, family unity, and parental support among refugee families after migration (Marley & Mauki, 2019).

Despite these considerations, previous research has focused mainly on the impact of parenting programs on the general population of parents and caregivers in Germany. To address this gap, the present study aimed to examine the impact of FMD on Arab families residing in Berlin, Germany. We expected parents to report improvements in well-being, reductions in distress and parental stress, and increased responsive (co-)parenting and paternal engagement after program participation. Exploratively, we examined increases in social support, availability of social and economic resources, and gender and youth empowerment. Drawing from emerging literature on the benefits of social support for parenting and parent support programs, we hypothesized that the changes in social support relate to the changes in parent characteristics (e.g., parental stress) and parenting behaviors.

Methodology

Research Design

The parents in the present study participated in groups of 5-10 in FMD parent meetings (or FMD groups). All groups were offered online except for one group with 10 individuals conducted in-person at a community center in Berlin. The participants completed the online survey with demographic questions and the outcome measures before the first session (time point 1, T1) and immediately after the last session (time point 2, T2). A trained research assistant was present to answer parents' questions in Arabic. The participants received an Amazon gift card (value 10 Euro) per completed survey.

Sample and Data Collection

One hundred thirty-five caregivers of children between 0 and 18 years old participated in this study. Most caregivers reported a nationality that differed from their country of residence (Germany) at the time of the survey. This sample was selected using non-random criteria based on geographical proximity (Berlin, Germany), availability, and resources such as access to a stable internet connection and web conferencing software. The program facilitators advertised the program implementation on social media and actively invited Arabic-speaking families to participate. Table 1 presents the key demographic characteristics of the participants.

Table 1. Sample Characteristics

	n	% or M (SD)	Min, Max
Caregiver sex			
Female	70	51.9%	
Male	65	48.1%	
Caregiver age (years) ^a		38.82 (8.01)	22, 65
Caregiver nationality			
Iraqi	1	0.7%	
Palestinian	19	14.1%	
Syrian	98	72.6%	
Tunisian	1	0.7%	
Multiple / Other	8	5.9%	
Missing	7	5.2%	
Number of children ^b		2.39 (1.26)	1, 6
Program participation			
Zero to 11-year-olds	80	59.3%	
Twelve to 18-year-olds	49	36.3%	
Missing	6	4.4%	
Person raising the children			
Participant alone	14	10.4%	
Participant and partner jointly	115	85.2%	
Missing	6	4.4%	
Time spent in Germany (years)		6.33 (2.84)	0.5, 23

Note. N = 135. ^a Information was missing for three caregivers. ^b Information was missing for five caregivers. Most caregivers had one (31.1%) or two (22.2%) children.

Several self-report questionnaires were used to measure maternal and paternal perceptions of well-being, stress, parenting, co-parenting behaviors, social support, empowerment, and availability of social and economic resources. All measures were administered in Arabic using an online survey. The questionnaires used to measure well-being, feelings of distress, father involvement, and social support had previously been used in a study in Beirut, Lebanon, to examine the impact of the Mother-Child Education Program (Ponguta et al., 2020).

Well-being, feelings of distress, and parental stress. Well-being was assessed using the WHO-5 well-being index of the World Health Organization (Topp et al., 2015). An example item reads: "I have felt calm and relaxed." Participants responded to the five items on a scale of 1 (*Never / at no time*) to 6 (*All of the time*). Good internal consistencies of the scale were found at T1 (Cronbach's $\alpha = .89$) and T2 (Cronbach's $\alpha = .92$). Feelings of distress were measured using the Individual Distress Scale (Ziadni et al., 2011). The 13 items ask participants to rate the affect and feelings they currently experience, using four response options ranging from 1 (*Not at all*) to 4 (*All the time*). Example items read: "Currently, to what extent do you feel unable to control the important things in your life?" and "Currently, to what extent do you feel worried?". The internal consistencies were excellent at T1 (Cronbach's $\alpha = .94$) and T2 (Cronbach's $\alpha = .94$). Finally, the stress in the role of a parent was measured using the Parental Stress Scale (PSS; Louie et al., 2018). The PSS includes 18 items and four subscales: (1) Rewards (six items; Example: "I am happy in my role as a parent."; Cronbach's

α at T1 = .80; T2 = .89); (2) Stressors (six items; Example: "The major source of stress in my life is my child(ren)."; Cronbach's α at T1 = .76; T2 = .81); (3) Lack of control (three items; Example: "I feel overwhelmed by the responsibility of being a parent."; Cronbach's α at T1 = .75; T2 = .78); and (4) Satisfaction (three items, Example: "I am satisfied as a parent."; Cronbach's α at T1 = .49; T2 = .56). The internal consistency of the total score of 18 items was good (Cronbach's α at T1 = .82; T2 = .85). Participants rated their feelings and perceptions about the experience of being a parent using five response options ranging from 1 (*Strongly disagree*) to 5 (*Strongly agree*).

Parenting and co-parenting. Multiple dimensions of parenting, co-parenting, and paternal engagement were assessed. Sensitive parenting was measured using four items (Verhoeven et al., 2016). A sample item reads: "I notice when my child is sad or doesn't feel good." Parents responded to each of the six items based on the frequency with which they engage in the respective behavior (1 = *Never*; 6 = *Always*). The internal consistency of the scale was good (Cronbach's α at T1 = .83; T2 = .89).

The participants also completed 17 items of the Multidimensional Assessment of Parenting (MAPS; Loiselle et al., 2021) measuring four dimensions of parenting: (1) Warmth/affectionate relationship (three items; Example: "I express affection by hugging, kissing, and holding my child."; Cronbach's α at T1 = .82; T2 = .89); (2) Supportiveness (three items; Example: "I encourage my child to talk about his/her troubles."; Cronbach's α at T1 = .82; T2 = .85); (3) Positive reinforcement (four items; Example: "If my child cleans his room, I will tell him/her how proud I am."; Cronbach's α at T1 = .73; T2 = .81); and (4) proactive parenting (seven items; Example: "I give reasons from my requests (such as "We must leave in 5 min, so it's time to clean up"; Cronbach's α at T1 = .75; T2 = .74). The response options ranged from 1 (*Never*) to 4 (*Always*). The total score of the four MAPS scales (labeled "responsive caregiving") was used in the analysis (Cronbach's α at T1 = .89; T2 = .92).

Co-parenting was measured using six items (Feinberg et al., 2012) tapping into two dimensions of co-parenting: Agreement (four items; Example: "My partner and I have the same goals for our child."; Cronbach's α at T1 = .74; T2 = .79) and division of labor (two items; Example, reversed coded: "My partner likes to play with our child and then leave dirty work to me."; Cronbach's α at T1 = .85; T2 = .81). The response options ranged from 0 (*Not true of us*) to 6 (*Very true of us*). The total score was analyzed (Cronbach's α at T1 = .78; T2 = .85).

Finally, we measured paternal engagement using the 20-item Father Involvement Questionnaire (FIQ; Hein et al., 2020; Ponguta et al., 2020). The FIQ measures perceptions of paternal engagement (1) with the child (nine items; Example maternal perceptions: "My husband speaks often with my child(ren)."; Cronbach's α at T1 = .84; T2 = .88), (2) with the mother (six items; Example: "My husband treats me well in front of our child(ren)."; Cronbach's α at T1 = .90; T2 = .91), and (3) in the community (five items; Example: "My husband enjoys spending time with other families."; Cronbach's α at T1 = .79; T2 = .80). Response options ranged from 1 (*Strongly disagree*) to 4 (*Strongly agree*).

Social support, availability of social and economic resources, and empowerment. Parents indicated their perceptions of social support using eight items of the Duke Social Support Index (DSSI; Koenig et al., 1993; Ponguta et al., 2020). One item asks the participants about the number of people (other than family members) in the local area they feel they can depend on or feel very close to (three response options ranging from 1 (*None*) to 3 (*More than two people*)). Parents indicated the frequency of perceived social support using six items (e.g., "When you are talking with your family, do you feel you are being listened to?"). The three response options ranged from 1 (*Hardly ever*) to 3 (*Most of the time*). The last item measured the degree of satisfaction with the relationships with one's family, ranging from 1 (*Very dissatisfied*) to 3 (*Satisfied*). The total scale score showed adequate internal consistency (Cronbach's α at T1 = .77; T2 = .85).

The availability of social and economic resources was measured using eight items (Chu et al., 2021). A sample item reads: "We ask neighbors for help and assistance." The four response options ranged from 1 (*Strongly disagree*) to 4 (*Strongly agree*). The internal consistency at both time points was good (Cronbach's α at T1 = .79; T2 = .88).

The attitudes toward gender and youth empowerment were measured using seven items (Bilandzic & Busselle, 2009), rated on four response options ranging from 1 (*Strongly disagree*) to 4 (*Strongly agree*). A sample item reads: "Women should have a say when important family decisions are made." The internal consistency of the scale was acceptable at both time points (Cronbach's α at T1 = .60; T2 = .69).

Analyzing of Data

The mean changes from T1 to T2 were analyzed using repeated-measures analysis of covariance (ANCOVA), controlling for the time between T1 and T2 (in days), caregiver sex, caregiver age, and the time spent in Germany (in years). We examined whether participants reported statistically significant changes in the outcome measures over time (within-subjects effect). We also examined whether the rates of change among female participants differed from the change reported by male participants (time-by-sex interaction). Manifest difference scores were estimated by subtracting the pre- (T1) from the post-intervention (T2) scores. This procedure replicates the approach taken by Álvarez et al. (2021) and can yield interpretable results given sufficient statistical power and repeated measures analysis (Thomas & Zumbo, 2011). Overall, most measures were completed by all participants, or only a minor extent of missing data was found

(typically one or two participants without valid data). The FIQ was the only measure that yielded missing data between 6 and 8.8% per time point, respectively. A complete case analysis was conducted in which cases without missing values were included in the respective repeated-measures ANCOVA. Key assumptions were tested (e.g., linearity of the association between pretest and posttest scores; no substantial outliers or extreme values above or below 1.5 times the interquartile range), and it was determined that an ANCOVA is appropriate. However, several outcome measures at T2 (e.g., well-being and parental stress) were not normally distributed. This was expected, given that FMD was explicitly designed for parents who experienced disruptive events in their biographies and are now seeking support to cope with the associated (psychosocial) challenges of these events. As such, the program does not involve a normative population of parents. Thus, it is expected that the variables are not entirely normally distributed. However, repeated-measures analysis of variance is robust against violation of normality assumptions in several simulation studies (e.g., Blanca et al., 2023; Schmider et al., 2010). Taken together, the repeated-measures ANCOVA is deemed appropriate for the present data.

Results

The time between T1 and T2 ranged from 9.89 to 91.36 days ($M = 37.8$, $SD = 27.86$). Table 2 shows the results of the repeated-measures ANCOVA. The sample showed small but statistically non-significant changes in the outcome measures over time. Male and female parents did not significantly differ in their perceived outcomes over time with one exception: parental stress. On average, female participants reported decreased overall parental stress (estimated marginal means at T1 = 2.38, T2 = 2.24) over time. In contrast, male participants, on average, reported an increase in parental stress (estimated marginal means at T1 = 2.24, T2 = 2.33).

Table 2. Analysis of Mean Changes Before and After Participation in FMD (repeated-measures ANCOVA).

	Time			Time-by-sex interaction			T1		T2		Difference T2 - T1	
	F	p	Partial η^2	F	p	Partial η^2	Female: M (SE)	Male: M (SE)	Female: M (SE)	Male: M (SE)	M (SD)	95%-CI (Lower, Upper)
Well-being	0.12	.73	.001	0.18	.67	.001	4.15 (.12)	4.34 (.13)	4.33 (.11)	4.59 (.12)	0.19 (0.89)	0.04, 0.34
Feelings of distress	2.25	.14	.018	0.82	.37	.007	1.89 (.07)	1.78 (.07)	1.77 (.07)	1.74 (.07)	-0.08 (0.49)	-0.16, 0.01
Parental stress – Total score	0.17	.68	.001	4.76	.03	.037	2.38 (.06)	2.24 (.07)	2.28 (.06)	2.33 (.07)	-0.01 (0.47)	-0.09, 0.07
Parental stress – Lack of rewards	0.01	.93	.000	0.00	.99	.000	1.60 (.58)	1.71 (.62)	1.63 (.72)	1.73 (.65)	-0.01 (0.65)	-0.12, 0.11
Parental stress – Stressors	0.32	.58	.003	7.23	.01	.055	2.86 (.09)	2.60 (.09)	2.60 (.09)	2.72 (.10)	-0.06 (0.78)	-0.19, 0.07
Parental stress – Lack of control	0.86	.36	.007	2.59	.11	.021	2.68 (.12)	2.20 (.13)	2.45 (.12)	2.28 (.13)	-0.05 (1.09)	-0.24, 0.14
Parental stress – Dissatisfaction	0.55	.46	.004	2.24	.14	.018	1.96 (.08)	1.93 (.09)	1.83 (.08)	1.99 (.09)	-0.05 (0.70)	-0.17, 0.06
Sensitive Parenting	0.22	.64	.002	0.07	.80	.001	5.18 (.12)	4.94 (.13)	5.22 (.11)	4.93 (.12)	0.02 (1.00)	-0.15, 0.19
Responsive caregiving	0.08	.77	.001	0.01	.99	.000	3.46 (.05)	3.39 (.06)	3.45 (.06)	3.40 (.06)	-0.01 (0.48)	-0.09, 0.07
Coparenting	0.19	.66	.002	1.99	.16	.018	4.02 (.16)	4.37 (.17)	4.15 (.18)	4.14 (.19)	-0.03 (1.31)	-0.26, 0.21
Social support	0.45	.50	.004	2.35	.13	.019	2.45 (.05)	2.56 (.05)	2.45 (.05)	2.46 (.06)	-0.05 (0.39)	-0.12, 0.02
Availability of social and economic resources	0.42	.52	.003	0.46	.50	.004	2.69 (.06)	2.75 (.06)	2.65 (.07)	2.78 (.07)	-0.03 (0.51)	-0.11, 0.06
Empowerment	0.10	.76	.001	0.01	.93	.000	3.18 (.05)	3.12 (.05)	3.09 (.05)	3.03 (.06)	-0.09 (0.38)	-0.16, -0.03
Father involvement – child	0.63	.43	.006	3.21	.08	.028	3.16 (.07)	3.43 (.07)	3.22 (.08)	3.30 (.08)	-0.05 (0.55)	-0.14, 0.05
Father involvement – mother	0.06	.81	.001	0.11	.74	.001	3.46 (.07)	3.67 (.08)	3.34 (.08)	3.59 (.09)	-0.08 (0.59)	-0.18, 0.03
Father involvement – community	0.54	.46	.005	2.43	.12	.022	2.80 (.08)	2.91 (.08)	2.60 (.09)	2.89 (.09)	-0.09 (0.66)	-0.21, 0.03

Notes. Ns ranged from 115 (father involvement – community) to 135. The analysis controlled for the time between T1 and T2 (in days), caregiver sex, caregiver age, and the time spent in Germany (in years). Estimated marginal means for males and females were estimated, controlling for the covariates. CI = Confidence Interval.

However, the magnitude of this moderated association by caregiver sex was small. Regarding subscales of the parental stress measure, the only significant association emerged for stressors. Female participants reported an overall decrease (estimated marginal means at T1 = 2.86, T2 = 2.60), and male participants reported a slight increase over time (estimated marginal means at T1 = 2.60, T2 = 2.72). The difference scores were overall relatively small and close to zero. The 95% confidence interval included zero for most outcomes except for well-being and empowerment. However, individual differences in the difference scores (as indicated by the standard deviations) suggested that parents differed in their rates of change from T1 to T2.

Associations between the change in social support and the change in outcome measures. We estimated a series of linear regression models to examine whether the change in social support from T1 to T2 predicted the change in outcome measures from T1 to T2. All regression models were controlled for the time between T1 and T2 (in days), how many years the participant has lived in Germany, caregiver sex, and caregiver age. Supplemental Table 1 shows the parameter estimates for seven outcomes that yielded statistically significant *F*-test values, indicating that the included predictor variables accounted for significant variation in the change scores. The results suggest that increases in the perceptions of social support were associated with reductions in parental stress ($\beta = -0.27$) and feelings of distress ($\beta = -0.47$) and increases in sensitive parenting ($\beta = 0.28$), responsive caregiving ($\beta = 0.37$), father involvement with the child ($\beta = 0.50$) and his spouse ($\beta = 0.48$), and co-parenting ($\beta = 0.33$).

Discussion

The findings of this impact evaluation demonstrate that participation in “Families Make the Difference” can potentially lower perceptions of parental stress among female Arab caregivers in Berlin, Germany. However, male caregivers reported a slight increase in perceptions of parental stress after participation in the program compared to before. Other evaluations of similar parenting programs implemented with Arab families have shown similar reductions in parental stress (e.g., in Lebanon; Lakkis et al., 2020; Ponguta et al., 2020). This finding is plausible given FMD’s strong emphasis on promoting parents’ strategies to manage stress and empowering parents and caregivers to navigate everyday stressors and hardship that many of them face due to experiences of displacement or migration. FMD may have provided new tools, strategies, or perspectives, especially to female participants, that make their caregiving role more manageable. It is, therefore, not surprising that female caregivers reported a reduction in parental stress, particularly given that the measure used in this study comprises several items about parents’ perceptions of stressors and overwhelming responsibilities in being a parent. At the same time, FMD encourages fathers to take an active role in parenting. This could increase their responsibility, which might increase their perceptions of stress, aligning with the level of maternal stress. Existing research demonstrated that mothers experience higher stress levels than fathers, albeit the effect size of this gender difference was small (see Pinquart, 2018, for a meta-analysis). Also, the fathers’ previous understanding of their role may have been challenged by participating in FMD. Adapting to new expectations may also explain, at least partly, the increase in parental stress. Furthermore, the program’s encouragement of father involvement may also make mothers feel relieved, decreasing their perceptions of parental stress. However, whether the observed changes in parental stress stem from role changes and new tools and strategies introduced by FMD remains speculative and should be evaluated in future research.

The FMD approach to bringing together caregivers to share personal experiences is a conduit for promoting assistance and encouragement in contexts where caregivers may have to rebuild their social and familial support networks. The participants also learn about the lifelong impact of early childhood adversity and toxic stress (Shonkoff et al., 2012) and how to mitigate the potentially harmful effects by providing nurturing care and responsive parenting. The opportunity to learn about ways to reduce one’s perceptions of stress can empower parents to take an active role in providing nurturing care for their children. However, we found that the relatively short time (an average of 37.8 days) between the measurement before and after program participation may not have been sufficient for FMD to yield changes in other outcomes, such as parenting and co-parenting behaviors. It appears that gains in knowledge (e.g., stress management strategies, child developmental milestones) are most likely to occur first, given FMD’s psychoeducational approach. Changes in tangible parenting behaviors may only unfold with more time and, therefore, more opportunities to practice the skills that parents have learned. Such a “sleeping effect” has been discussed for different types of parenting programs (Bornstein et al., 2022). For example, a systematic review and meta-analysis of parenting interventions for disruptive child behavior that included 40 randomized controlled trials with follow-up assessments yielded that some interventions show sleeping effects (van Aar et al., 2017). However, the circumstances under which sleeping effects occur are not fully understood. Thus, a follow-up assessment (e.g., six months post-intervention) is required to determine whether parents implement strategies and skills in their everyday parenting.

Another key finding of this study was that increases in the perceptions of social support over time were associated with reductions in parental stress and feelings of distress and increases in sensitive parenting, a composite measure of responsive caregiving, paternal engagement with the child and his spouse, and increases in co-parenting. This finding confirms and expands, in part, the findings of Álvarez et al. (2021). On average, the participants did not report significant changes in social support over time. However, the variability in change scores implies individual differences in the perceptions of social support after participating in the program compared to before. Some of the participating

caregivers may have decided to participate in FMD to solicit social support in child-rearing and to address their socioemotional needs as well as the needs of their children. The changes in social support may be a protective factor in reducing parental stress and expanding the opportunities for caregivers to fully engage in responsive caregiving and co-parenting. What remains unclear, however, is whether FMD provided parents with a supportive network of other caregivers with similar experiences and trained facilitators or if parents sought support from other sources. The type of social support (e.g., emotional, instrumental) could also not be reliably differentiated.

Conclusion

The present evaluation of FMD has shown that female participants benefit from the participation, given the reduced perceptions of parental stress after the program. However, we also noted an increase in male caregivers, which might be associated with new perspectives on their family role. This discrepancy also highlights the complexity of parental roles and the challenges fathers face in (re-)defining their responsibilities within the family context. Hence, future research should focus on the differential needs of female and male caregivers, taking particular care to ensure that no one is overwhelmed. Furthermore, our study showed that increases in the perceptions of social support were associated with some outcome variables (e.g., reductions in parental stress and increases in sensitive parenting), therefore underlying the importance and benefits of social networks for migrant families.

Recommendations

Future implementation of programs such as FMD should aim to tailor intervention delivery to the needs and preferences of the target group while maintaining a comparable overall program duration. Moreover, child outcomes were not measured in this study. Future impact evaluations should include child outcomes such as cognitive and language outcomes, emotional and behavioral difficulties, and prosocial behavior (Jeong et al., 2021; Lakkis et al., 2020). Future studies should also include observations of parenting behavior and dyadic measures of co-parenting to better capture the family dynamics. Future studies should also examine mutual support-giving dynamics and properties of groups of parents who participate in FMD that can provide a sense of social support (Thoits, 1995). In this regard, the potential moderating effects of the delivery format (online compared to in-person) should also be examined, as the format may account for differences in program effects and perceptions of social support. Future studies should collect data from a larger sample to model error-free constructs and derive more reliable estimates of the association between the change in social support and parenting characteristics over time. Regarding practical recommendations, this study highlights the benefit of group facilitators with similar lived experiences and biographies as the participating parents. Based on these experiences, the facilitators may provide more tailored support to the participants and encourage parents to support each other based on shared needs and priorities. Facilitators must also know the local communities so they can refer parents to other support services if needed. Concerning program delivery, each program session should allow for sufficient time for parents to solicit and offer support related to the specific topic of discussion (e.g., how to find adequate housing and institutional child care). Moreover, the overall program duration should be long enough (e.g., weekly or biweekly sessions) so that parents can use the strategies they learned and leverage the support they received during the program.

Limitations

Several limitations of this impact evaluation must be acknowledged. First, we could not include a control group in the design of this study. Thus, we cannot adequately appraise whether the changes in parental stress result from the participation in FMD or are fluctuations in Arab families that are typical for the time between both measurements (at most three months). Therefore, future research should examine the impact of FMD by comparing an intervention group to a control group. Ideally, such an evaluation would include both a non-treatment control group and a control group that receives an alternative program comparable to the intervention program, for example, in terms of the number of sessions, duration, and level of involvement of the researcher. Second, the program dosage, specifically the frequency of sessions, varied across the study period. This was mainly due to the participants' preferences. Some groups preferred to cover the entire program over nine days with one daily session. Other groups opted to meet twice a week for a longer program duration. Adapting the program delivery based on the participants' preferences contributed to the retention of parents in the program. However, the variability in the program dosage across settings means that the time parents had to reflect on what they have learned and apply it in their everyday routines varied substantially. Third, the operationalization of social support should be expanded to differentiate types of social support (e.g., emotional support, instrumental support). Fifth, we used manifest change scores (i.e., the difference between scale scores at T1 and T2) in the regression analysis. Latent difference score models are a more robust alternative that considers measurement error (Castro-Schilo & Grimm, 2017). However, our sample size did not provide sufficient statistical power to meet the rigorous measurement invariance requirements of latent difference score models. Finally, in 2023, IRC revised and redesigned the FMD handbook, consolidating the two handbooks into one comprehensive resource. The revision integrated feedback from focus groups with former participating parents and supervision sessions with facilitators. Four new thematic units were introduced, covering topics such as multilingualism, empowerment through addressing discrimination and racism, social media, and bullying. The goal was to create a clear, intuitive, and practical resource

for facilitators, allowing them to flexibly adapt to each parent group's evolving needs and priorities. The revised handbook includes a variety of themes, activities, and interactive methods to engage parents and address their concerns effectively. The present study evaluated the previous version of the program that included one handbook per target age group (0-11-year-olds and 12-18-year-olds). Although the core curriculum and program activities of the previous version are the same as those in the revised version, more research is needed to understand the impact of the revised handbook.

Ethics Statements

The Ethics Review Committee of the Department of Education and Psychology, Freie Universität Berlin, approved this study. The participants provided their informed consent before filling in the online survey.

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Conflict of Interest

All authors confirm no conflict of interest.

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Hein: Concept and design, data acquisition, data analysis / interpretation, drafting manuscript, statistical analysis, securing funding, admin, supervision, final approval. Schuster: Concept and design, data acquisition, writing, editing/reviewing, admin, supervision. Alassi: Editing/reviewing, technical support. Mohamad: Editing/reviewing, technical support.

References

- Álvarez, M., Byrne, S., & Rodrigo, M. J. (2021). Social support dimensions predict parental outcomes in a Spanish early intervention program for positive parenting. *Children and Youth Services Review*, 121, Article 105823. <https://doi.org/10.1016/j.childyouth.2020.105823>
- Ayala-Nunes, L., Nunes, C., & Lemos, I. (2016). Social support and parenting stress in at-risk Portuguese families. *Journal of Social Work*, 17(2), 207-225. <https://doi.org/10.1177/1468017316640200>
- Bennett, C., Barlow, J., Huband, N., Smailagic, N., & Roloff, V. (2013). Group-based parenting programs for improving parenting and psychosocial functioning: A systematic review. *Journal of the Society for Social Work and Research*, 4(4), 300-332. <https://doi.org/10.5243/jsswr.2013.20>
- Berry, J. O., & Jones, W. H. (1995). The Parental Stress Scale: Initial psychometric evidence. *Journal of Social and Personal Relationships*, 12(3), 463-472. <https://doi.org/10.1177/0265407595123009>
- Bilandzic, H., & Busselle, R. (2009). *Pilot study: Audience perceptions and effects of the entertainment-education drama "The Station"* (Research report). Search for Common Ground. <https://bit.ly/3DFL0nK>
- Blanca, M. J., Arnau, J., Javier García-Castro, F., Alarcón, R., & Bono, R. (2023). Non-normal data in repeated measures ANOVA: Impact on type I error and power. *Psicothema*, 35(1), 21-29. <https://doi.org/10.7334/psicothema2022.292>
- Bornstein, M. H., Cluver, L., Deater-Deckard, K., Hill, N. E., Jager, J., Krutikova, S., Lerner, R. M., & Yoshikawa, H. (2022). The future of parenting programs: I Design. *Parenting*, 22(3), 201-234. <https://doi.org/10.1080/15295192.2022.2087040>

- Bulford, E., Fogarty, A., Giallo, R., Brown, S., Szwarc, J., & Riggs, E. (2022). Risk and protective factors experienced by fathers of refugee background during the early years of parenting: A qualitative study. *International Journal of Environmental Research and Public Health*, 19(11), Article 6940. <https://doi.org/10.3390/ijerph19116940>
- Castro-Schilo, L., & Grimm, K. J. (2017). Using residualized change versus difference scores for longitudinal research. *Journal of Social and Personal Relationships*, 35(1), 32-58. <https://doi.org/10.1177/0265407517718387>
- Chu, A. M. Y., Tsang, J. T. Y., Tiwari, A., Yuk, H., & So, M. K. P. (2021). Measuring family resilience of Chinese family caregivers: Psychometric evaluation of the Family Resilience Assessment Scale. *Family Relations*, 71(1), 130-146. <https://doi.org/10.1111/fare.12601>
- Cooley, M. E., Veldorale-Griffin, A., Petren, R. E., & Mullis, A. K. (2014). Parent-child interaction therapy: A meta-analysis of child behavior outcomes and parent stress. *Journal of Family Social Work*, 17(3), 191-208. <https://doi.org/10.1080/10522158.2014.888696>
- de Santis, L., de Carvalho, T. R., de Lima Guerra, L. L., dos Santos Rocha, F., & Barham, E. J. (2020). Supporting fathering: A systematic review of parenting programs that promote father involvement. *Trends in Psychology*, 28, 302-320. <https://doi.org/10.9788/s43076-019-00008-z>
- El-Khani, A., Ulph, F., Peters, S., & Calam, R. (2016). Syria: The challenges of parenting in refugee situations of immediate displacement. *Intervention: Journal of Mental Health and Psychosocial Support in Conflict Affected Areas*, 14(2), 99-113. <https://doi.org/10.1097/wtf.0000000000000118>
- Fang, Y., Luo, J., Boele, M., Windhorst, D., van Grieken, A., & Raat, H. (2024). Parent, child, and situational factors associated with parenting stress: A systematic review. *European Journal of Child and Adolescent Psychiatry*, 33(6), 1687-1705. <https://doi.org/10.1007/s00787-022-02027-1>
- Feinberg, M. E., Brown, L. D., & Kan, M. L. (2012). A multi-domain self-report measure of coparenting. *Parenting: Science and Practice*, 12(1), 1-21. <https://doi.org/10.1080/15295192.2012.638870>
- Gillespie, S., Banegas, J., Maxwell, J., Chan, A. C. Y., Darawshy, N. A., Wasil, A. R., Marsalis, S., & Gewirtz, A. (2022). Parenting interventions for refugees and forcibly displaced families: A systematic review. *Clinical Child and Family Psychology Review*, 25(2), 395-412. <https://doi.org/10.1007/s10567-021-00375-z>
- Gonzalez, J. C., Klein, C. C., Barnett, M. L., Schatz, N. K., Garoosi, T., Chacko, A., & Fabiano, G. A. (2023). Intervention and implementation characteristics to enhance father engagement: A systematic review of parenting interventions. *Clinical Child and Family Psychology Review*, 26(2), 445-458. <https://doi.org/10.1007/s10567-023-00430-x>
- Hamari, L., Konttila, J., Merikukka, M., Tuomikoski, A. M., Kouvonen, P., & Kurki, M. (2022). Parent support programmes for families who are immigrants: A scoping review. *Journal of Immigrant and Minority Health*, 24(2), 506-525. <https://doi.org/10.1007/s10903-021-01181-z>
- Hein, S., Bick, J., Issa, G., Aoude, L., Maalouf, C., Awar, A., Nourallah, S., Zonderman, A. L., Householder, S., Katsoyich, L., Khoshnood, K., Moore, C., Salah, R., Britto, P. R., Leckman, J. F., & Ponguta, L. A. (2020). Maternal perceptions of father involvement among refugee and disadvantaged families in Beirut, Lebanon. *PLoS One*, 15(3), Article e0229670. <https://doi.org/10.1371/journal.pone.0229670>
- Henry, J. B., Julion, W. A., Bounds, D. T., & Sumo, J. (2020). Fatherhood matters: An integrative review of fatherhood intervention research. *Journal of School Nursing*, 36(1), 19-32. <https://doi.org/10.1177/1059840519873380>
- Holmes, E. K., Egginton, B. R., Hawkins, A. J., Robbins, N. L., & Shafer, K. (2020). Do responsible fatherhood programs work? A comprehensive meta-analytic study. *Family Relations*, 69(5), 967-982. <https://doi.org/10.1111/fare.12435>
- Jeong, J., Franchett, E. E., Ramos de Oliveira, C. V., Rehmani, K., & Yousafzai, A. K. (2021). Parenting interventions to promote early child development in the first three years of life: A global systematic review and meta-analysis. *PLoS Med*, 18(5), Article e1003602. <https://doi.org/10.1371/journal.pmed.1003602>
- Koenig, H. G., Westlund, R. E., George, L. K., Hughes, D. C., Blazer, D. G., & Hybels, C. (1993). Abbreviating the Duke Social Support Index for use in chronically ill elderly individuals. *Psychosomatics*, 34(1), 61-69. [https://doi.org/10.1016/S0033-3182\(93\)71928-3](https://doi.org/10.1016/S0033-3182(93)71928-3)
- Lakkis, N. A., Osman, M. H., Aoude, L. C., Maalouf, C. J., Issa, H. G., & Issa, G. M. (2020). A pilot intervention to promote positive parenting in refugees from Syria in Lebanon and Jordan. *Frontiers in Psychiatry*, 11, Article 257. <https://doi.org/10.3389/fpsy.2020.00257>
- Lee, I. S., & Kim, E. (2022). Effects of parenting education programs for refugee and migrant parents: A systematic review and meta-analysis. *Child Health Nursing Research*, 28(1), 23-40. <https://doi.org/10.4094/chnr.2022.28.1.23>

- Li, N., Peng, J., & Li, Y. (2021). Effects and moderators of Triple P on the social, emotional, and behavioral problems of children: Systematic review and meta-analysis. *Frontiers in Psychology*, 12, Article 709851. <https://doi.org/10.3389/fpsyg.2021.709851>
- Lim, N., O'Reilly, M., Russell-George, A., & Londono, F. V. (2023). A meta-analysis of parenting interventions for immigrants. *Prevention Science*, 24(6), 1152-1173. <https://doi.org/10.1007/s1121-022-01488-9>
- Liu, S. W., Zhai, F., & Gao, Q. (2020). Parental stress and parenting in Chinese immigrant families: The mediating role of social support. *Child & Family Social Work*, 25(S1), 135-148. <https://doi.org/10.1111/cfs.12734>
- Loiselle, R., Parent, J., Georgeson, A. R., Thissen, D., Jones, D. J., & Forehand, R. (2021). Validation of the Multidimensional Assessment of Parenting: An application of item response theory. *Psychological Assessment*, 33(9), 803-815. <https://doi.org/10.1037/pas0001019>
- Louie, A. D., Cromer, L. D., & Berry, J. O. (2018). Assessing parenting stress: Review of the use and interpretation of the parental stress scale. *The Family Journal*, 25(4), 359-367. <https://doi.org/10.1177/1066480717731347>
- Lundahl, B. W., Tollefson, D., Risser, H., & Lovejoy, M. C. (2008). A meta-analysis of father involvement in parent training. *Research on Social Work Practice*, 18(2), 97-106. <https://doi.org/10.1177/1049731507309828>
- Marley, C., & Mauki, B. (2019). Resilience and protective factors among refugee children post-migration to high-income countries: A systematic review. *European Journal of Public Health*, 29(4), 706-713. <https://doi.org/10.1093/eurpub/cky232>
- Masarik, A. S., & Conger, R. D. (2017). Stress and child development: A review of the Family Stress Model. *Current Opinion in Psychology*, 13, 85-90. <https://doi.org/10.1016/j.copsyc.2016.05.008>
- Masarik, A. S., Fritz, H., & Lazarevic, V. (2022). Stress and resilience among resettling refugee youth: An illustrative review and new applications for the family stress model. *Journal of Family Theory & Review*, 14(2), 207-232. <https://doi.org/10.1111/jftr.12454>
- Mytton, J., Ingram, J., Manns, S., & Thomas, J. (2014). Facilitators and barriers to engagement in parenting programs: A qualitative systematic review. *Health Education & Behavior*, 41(2), 127-137. <https://doi.org/10.1177/1090198113485755>
- Norris, F. H., & Kaniasty, K. (1996). Received and perceived social support in times of stress: A test of the social support deterioration deterrence model. *Journal of Personality and Social Psychology*, 71(3), 498-511. <https://doi.org/10.1037/0022-3514.71.3.498>
- Panter-Brick, C., Burgess, A., Eggerman, M., McAllister, F., Pruett, K., & Leckman, J. F. (2014). Practitioner review: Engaging fathers--recommendations for a game change in parenting interventions based on a systematic review of the global evidence. *Journal of Child Psychology and Psychiatry*, 55(11), 1187-1212. <https://doi.org/10.1111/jcpp.12280>
- Pinquart, M. (2018). Parenting stress in caregivers of children with chronic physical condition--A meta-analysis. *Stress & Health*, 34(2), 197-207. <https://doi.org/10.1002/smi.2780>
- Ponguta, L. A., Issa, G., Aoudeh, L., Maalouf, C., Hein, S., Bick, J., Zonderman, A. L., Kastovich, L., Khoshnood, K., Awar, A., Nourallah, S., Householder, S., Moore, C. C., Salah, R., & Leckman, J. F. (2020). Effects of the Mother-Child Education Program on parenting stress and disciplinary practices among refugee and other marginalized communities in Lebanon: A pilot randomized controlled trial. *Journal of the American Academy of Child and Adolescent Psychiatry*, 59(6), 727-738. <https://doi.org/10.1016/j.jaac.2019.12.010>
- Rao, N., Kumar, J., Weeks, E. A., Self-Brown, S., Wilging, C. E., O'Connor, M. H., & Whitaker, D. J. (2023). "It's hard because it's something new": Challenges for forcibly displaced parents upon resettlement. *Journal of Children's Services*, 19(1), 1-19. <https://doi.org/10.1108/JCS-01-2023-0002>
- Schmider, E., Ziegler, M., Danay, E., Beyer, L., & Bühner, M. (2010). Is it really robust? Reinvestigating the robustness of ANOVA against violations of the normal distribution assumption. *Methodology*, 6(4), 147-151. <https://doi.org/10.1027/1614-2241/a000016>
- Shonkoff, J. P., Garner, A. S., The Committee on Psychosocial Aspects of Child and Family Health, Committee on Early Childhood, Adoption, and Dependent Care, Section on Developmental and Behavioral Pediatrics, Siegel, B. S., Dobbins, M. I., Earls, M. F., Garner, A. S., McGuinn, L., Pascoe, J., & Wood, D. L. (2012). The lifelong effects of early childhood adversity and toxic stress. *Pediatrics*, 129(1), e232-e246. <https://doi.org/10.1542/peds.2011-2663>
- Stewart, M., Kushner, K. E., Dennis, C., Kariwo, M., Letourneau, N., Makumbe, K., Makwarimba, E., & Shizha, E. (2017). Social support needs of Sudanese and Zimbabwean refugee new parents in Canada. *International Journal of Migration, Health and Social Care*, 13(2), 234-252. <https://doi.org/10.1108/ijmhsc-07-2014-0028>

- Stewart, M., Spitzer, D. L., Kushner, K. E., Shizha, E., Letourneau, N., Makwarimba, E., Dennis, C.-L., Kariwo, M., Makumbe, K., & Edey, J. (2018). Supporting refugee parents of young children: "knowing you're not alone". *International Journal of Migration, Health and Social Care*, 14(1), 15-29. <https://doi.org/10.1108/ijmhsc-04-2016-0018>
- Tehrani, H. D., Yamini, S., & Vazsonyi, A. T. (2024). Effects of parenting program components on parental stress: A systematic review and component network meta-analysis. *Journal of Family Psychology*, 38(2), 320-332. <https://doi.org/10.1037/fam0001161>
- Thoits, P. A. (1995). Stress, coping, and social support processes: Where are we? What next? *Journal of Health and Social Behavior*, (Extra Issue), 53-79. <https://doi.org/10.2307/2626957>
- Thomas, D. R., & Zumbo, B. D. (2011). Difference scores from the point of view of reliability and repeated-measures ANOVA. *Educational and Psychological Measurement*, 72(1), 37-43. <https://doi.org/10.1177/0013164411409929>
- Topp, C. W., Ostergaard, S. D., Sondergaard, S., & Bech, P. (2015). The WHO-5 Well-Being Index: A systematic review of the literature. *Psychotherapy and Psychosomatics*, 84(3), 167-176. <https://doi.org/10.1159/000376585>
- van Aar, J., Leijten, P., Orobio de Castro, B., & Overbeek, G. (2017). Sustained, fade-out or sleeper effects? A systematic review and meta-analysis of parenting interventions for disruptive child behavior. *Clinical Psychology Review*, 51, 153-163. <https://doi.org/10.1016/j.cpr.2016.11.006>
- Verhoeven, M., Deković, M., Bodden, D., & van Baar, A. L. (2016). Development and initial validation of the comprehensive early childhood parenting questionnaire (CECPAQ) for parents of 1–4 year-olds. *European Journal of Developmental Psychology*, 14(2), 233-247. <https://doi.org/10.1080/17405629.2016.1182017>
- Zhang, C., Cubbin, C., & Ci, Q. (2016). Parenting stress and mother–child playful interaction: The role of emotional support. *Journal of Family Studies*, 25(2), 101-115. <https://doi.org/10.1080/13229400.2016.1200113>
- Ziadni, A., Hammoudeh, W., Abu Rmeileh, N. M. E., Hogan, D., Shannon, H., & Giacaman, R. (2011). Sources of human insecurity in post-war situations: The case of Gaza. *Journal of Human Insecurity*, 7(3), 23-36. <https://doi.org/10.3316/JHS0703023>